



Pathology Results

Name: _____ MRN: _____
DOB: _____
Sex: _____

Collected: _____ Case#: _____
Date Rec'd: _____
Ordering MD: _____
Physician: _____ Status: FINAL
Service: _____
Related Reports: _____
Surgical Pathology

Patient Name
MR#: _____
Specimen #

Final Diagnosis

- A. Right ureter, biopsy:
- No evidence of carcinoma or high grade dysplasia
- B. Left ureter, biopsy:
- No evidence of carcinoma or high grade dysplasia
- C. Left external and internal iliac lymph nodes, dissection:
- Eight lymph nodes with no evidence of metastatic carcinoma (0/8)
- D. Left external and internal iliac lymph nodes, dissection:
- Two lymph nodes with no evidence of metastatic carcinoma (0/2)
- E. Left obturator lymph nodes, dissection:
- Six lymph nodes with no evidence of metastatic carcinoma (0/6)
- F. Left common iliac lymph nodes, dissection:

ICD-O-3

Carcinoma, urothelial, NOS

8120/3

Site: bladder, wall, NOS

C67.9

8120/3
R0

See TCGA Pathologic Diagnosis
Discrepancy Form.

Criteria	Yes	No
Diagnosis Discrepancy	☒	☒
Primary Tumor Site Discrepancy	☒	☒
Site/AA Discrepancy	☒	☒
Prior Malignancy History	☒	☒
Diagnosis Synchronicity Primary/Noted	☒	☒
Case Is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	MB	8/20/12

**** This report is a compilation of information originating from multiple source systems. ****

Pathology Results

Name:

MRN:

DOB:

Sex:

- Eleven lymph nodes with no evidence of metastatic carcinoma (0/11)

G. Presacral lymph nodes, dissection:

- Four lymph nodes with no evidence of metastatic carcinoma (0/4)

H. Bilateral common iliac lymph nodes, dissection:

- Two lymph nodes with no evidence of metastatic carcinoma (0/2)

I. Paraaortic and paracaval lymph nodes, dissection:

- Eight lymph nodes with no evidence of metastatic carcinoma (0/8)

J. Right common iliac lymph nodes, dissection:

- Six lymph nodes with no evidence of metastatic carcinoma (0/6)

K. Right external and internal iliac lymph nodes, dissection:

- Seven lymph nodes with no evidence of metastatic carcinoma (0/7)

L. Right obturator lymph nodes, dissection:

- Fourteen lymph nodes with no evidence of metastatic carcinoma (0/14)

M. Urinary bladder, ureter stumps, prostate and seminal vesicles, radical cystoprostatectomy:

- High grade urothelial (transitional cell) carcinoma with extensive squamous differentiation,

invasive into perivesical adipose tissue, (see note)

- The surgical resection margins are free of carcinoma or high grade dysplasia

- Prostate with no evidence of carcinoma

- See CAP staging protocol below for full report

N. Appendix, appendectomy:

- Appendix with no significant pathologic findings

CAP Staging Protocol for Carcinoma of the Urinary Bladder

Specimen: Bladder and prostate

**** This report is a compilation of information originating from multiple source systems. ****

Pathology Results

Name:

MRN:

DOB:

Sex:

Procedure: Radical cystoprostatectomy

Tumor Site: Right lateral and posterior walls

Tumor Size: 3.3 cm in greatest dimension

Histologic Type: Urothelial (transitional cell) carcinoma with extensive squamous differentiation (see note)

Associated Epithelial Lesions: None identified

Histologic Grade: Urothelial Carcinoma (WHO :UP): High-grade

Microscopic Tumor Extension: Perivesical fat

Margins: Margins uninvolved by invasive or in situ carcinoma

Lymph-Vascular Invasion: Not identified

Pathologic Staging (pTNM):

Primary Tumor (pT):

pT3b: Tumor invades perivesical tissue, macroscopically

Regional Lymph Nodes (pN):

pN0: No lymph node metastasis

Specify: Number examined: 68, Number involved: 0

Distant Metastasis (pM): Not applicable

Ancillary Studies: Block M7 will be sent for ancillary studies

The above pathology diagnosis incorporates data elements as outlined in the CAP Cancer Protocols and Checklists which are based on the AJCC/UICC TNM, 7th edition.

Note

**** This report is a compilation of information originating from multiple source systems. ****

Pathology Results

Name: _____

MRN: _____

DOB: _____

Sex: _____

The patient's previous outside TURBT material _____ was reviewed, and in these sections the tumor displays marked areas of sarcomatous differentiation with multiple heterologous histomorphologies. The current cystectomy specimen displays a high grade urothelial carcinoma with very extensive squamous differentiation, large zones of necrosis, very poorly-differentiated areas, and focal sarcomatous areas, although no heterologous components are seen.

This case was reviewed at the _____
_____ the above diagnosis represents a consensus opinion.

Clinical History

Bladder cancer

Radical cystectomy, lymph node dissection, ileal conduit creation, all other indicated procedures

Source:

A: Right ureter

B: Left ureter

C: Left external and internal iliac lymph nodes

D: Left external and internal iliac Lymph node

E: Left obturator Lymph node

F: Left common iliac Lymph node

G: Presacral Lymph node

H: Bilateral common iliac Lymph node

I: Preaortal and precaval Lymph node

J: Right common iliac Lymph node

K: Right external and internal iliac Lymph node

L: Right obturator Lymph node

M: Bladder and prostate

N: Appendix OT Incidental

Gross Description

Specimen A is received in formalin and designated 'right ureter' and consists of a gray-white tubular soft tissue fragment measuring 1.3 cm in length and 0.4 cm in diameter. The tube is marked with a suture tag at what is indicated to be

**** This report is a compilation of information originating from multiple source systems. ****

Pathology Results

Name: _____

MRN: _____

DOB: _____

Sex: _____

the distal margin. A cross section of the proximal margin was frozen and submitted, A1.

Specimen B is received fresh and designated 'left ureter' and consists of gray-white tubular soft tissue fragment measuring 1.3 cm in length and 0.7 cm in diameter. The tissue is received with a suture tag at what is designated distal margin. On section, the tissue is seen to contain a double lumen with separate walls and contain with a common sheet. The proximal resection margin is frozen and subsequently submitted, B1.

Specimen C is received fresh and designated 'left external and internal iliac lymph nodes' and consists of an irregular portion of focally hemorrhagic lobulated adipose tissue measuring 12.0 x 4.0 x 1.0 cm. Dissected from the adipose tissue are eight lymph node structures ranging from 0.3 cm to 2.0 cm. The nodes were entirely frozen and are subsequently submitted as follows:

- C1 three whole lymph nodes
- C2 one bisected lymph node
- C3 one bisected lymph node
- C4 one bisected lymph node
- C5 one bisected lymph node
- C6 one bisected lymph node

Specimen D is received in formalin and designated 'left external and internal iliac lymph nodes' and consists of an aggregate of several fragments of focally hemorrhagic lobulated adipose tissue measuring 3.6 x 3.0 x 0.8 cm. Sectioning through the adipose tissue reveals two lymph node structures ranging from 0.7 cm to 2.2 cm. Nodes are submitted as follows:

- D1 smaller node bisected
- D2 larger node bisected

Specimen E is received in formalin and designated 'left obturator lymph nodes' and consists of an aggregate of multiple fragments of focally hemorrhagic lobulated adipose tissue. The fragments measure in aggregate 5.3 x 4.2 x 1.5 cm. On section, the adipose tissue fragments reveal eight apparent lymph node structures ranging from 0.4 cm to 2.5 cm. The nodes are entirely submitted as follows:

**** This report is a compilation of information originating from multiple source systems. ****

Pathology Results

Name:

MRN:

DOB:

Sex:

- E1 single bisected node
- E2 single bisected node
- E3 single bisected node
- E4 five whole nodes

Specimen F is received in formalin and designated 'left common iliac lymph nodes' and consists of an aggregate of multiple focally hemorrhagic fragments of lobulated adipose tissue measuring 4.7 x 3.2 x 1.5 cm. On section, adipose tissue reveals 11 apparent lymph node structures ranging from 0.5 cm to 2.0 cm.

The nodes are entirely submitted as follows:

- F1 one bisected lymph node
- F2 - one bisected lymph node
- F3 one bisected lymph node
- F4 one bisected lymph node
- F5 one bisected lymph node
- F6 one bisected lymph node
- F7 five whole lymph nodes

Specimen G is received in formalin and designated 'presacral lymph nodes' and consists of an aggregate of several fragments of focally hemorrhagic lobulated adipose tissue measuring 4.5 x 3.5 x 1.0 cm. Sectioning through the adipose tissue reveals six apparent lymph node structures ranging from 0.3 cm to 0.7 cm.

Nodes are entirely submitted as follows:

- G1 three whole lymph nodes
- G2 three whole lymph nodes

Specimen H is received in formalin and designated 'bilateral common iliac lymph nodes' and consists of an aggregate of focally hemorrhagic lobulated adipose tissue. The fragments measure 4.1 x 2.8 x 0.9 cm. Sectioning through the adipose tissue reveals three apparent lymph node structures ranging from 0.5 cm to 1.1 cm. Nodes are entirely submitted as follows:

- H1 two whole nodes
- H2 one bisected node

Specimen I is received in formalin and designated 'pre-aortal and pre-caval lymph nodes' and consists of several fragments of focally hemorrhagic lobulated

**** This report is a compilation of information originating from multiple source systems. ****

Pathology Results

Name:

MRN:

DOB:

Sex:

adipose tissue measuring in aggregate 4.3 x 2.7 x 1.2 cm. Sectioning through the adipose tissue reveals eight apparent lymph node structures ranging from 0.3 cm to 1.5 cm. The nodes are entirely submitted as follows:

- I1 single bisected node
- I2 single bisected node
- I3 single bisected node
- I4 single bisected node
- I5 single trisected node
- I6 three whole nodes

Specimen J is received in formalin and designated 'right common iliac lymph nodes' and consists of two fragments of focally hemorrhagic lobulated adipose tissue measuring in aggregate 2.5 x 2.0 x 0.8 cm. Sectioning through the adipose tissue reveals five apparent lymph nodes ranging from 0.5 cm to 1.1 cm. Nodes are entirely submitted as follows:

- J1 single bisected node
- J2 single bisected node
- J3 three whole nodes

Specimen K is received in formalin and designated 'right external and internal iliac lymph nodes' and consists of an aggregate of several fragments of focally hemorrhagic lobulated adipose tissue. The fragments measure in aggregate 5.3 x 3.2 x 1.2 cm. Sectioning through the adipose tissue reveals seven apparent lymph node structures ranging from 0.3 cm to 1.4 cm. Nodes are entirely submitted as follows:

- K1 single bisected node
- K2 single bisected node
- K3 single bisected node
- K4 four whole nodes

Specimen L is received in formalin and designated 'right obturator lymph nodes' and consists of an aggregate of multiple fragments of focally hemorrhagic lobulated adipose tissue. The fragments measure in aggregate 5.3 x 3.5 x 1.2 cm. Cross section of the adipose tissue reveal 14 apparent lymph node structures ranging from 0.3 cm to 2.5 cm. Entirely submitted as follows:

- L1 single bisected node

**** This report is a compilation of information originating from multiple source systems. ****

Pathology Results

Name:

MRN:

DOB:

Sex:

- L2 single bisected node
- L3 single bisected node
- L4 single bisected node
- L5 single bisected node
- L6 five whole nodes
- L7 four whole nodes

Specimen M

Type of specimen: Urinary bladder, ureter stumps, prostate, seminal vesicles

The patient's name and case number on the specimen container match the accompanying paperwork and cassettes.

Procedure: Radical cystoprostatectomy

Fixative: In formalin

Dimensions: Bladder, SI 6.0 cm; right to left 5.0 cm; AP 3.5 cm; detached ureter 2.5 cm in length and 0.2 cm in diameter; attached left ureter 2.0 cm in length and 0.6 cm in diameter and reveals a double lumen. Prostate measures 2.8 cm SI, 3.2 cm right to left, and 2.3 cm AP.

Description of findings:

The bladder has a moderate amount of attached perivesicular adipose tissue. Upon opening the bladder, the inner lining reveals a large slightly elevated tumor mass occupying portions of right lateral wall and extending to include portion of the posterior wall. Tumor measures 3.3 cm AP and 2.6 cm right to left. Portions of the tumor have rolled margins. The center of the tumor has a soft friable necrotic appearance. The right ureteral orifice has been mechanically displaced by the tumor mass. The left ureter enters the bladder with two separate distinct ureteral orifices, corresponding to the duplication of left ureter. Sectioning through the bladder wall reveals the tumor to have invaded the full thickness of the bladder wall and into the attached perivesicular adipose tissue. The tumor has a maximum thickness of 1.3 cm. The remaining bladder mucosa has a mildly edematous gray-white appearance. No additional mass or lesions are grossly identified. Sectioning through the prostate reveals gray-white lobulated spongy cut surfaces. No masses are grossly identified.

Ink code:

Anterior surface blue

Posterior black

Right prostate, left orange

**** This report is a compilation of information originating from multiple source systems. ****

Pathology Results

Name:

MRN:

DOB:

Sex:

Left prostate inked green

Right lateral resection margin of the bladder red

Section key:

M1 longitudinal section of left ureteral orifice including both lumen

M2 right ureteral orifice, longitudinal

M3 tumor with inked posterior margin

M4 tumor right lateral margin

M5, M6 additional sections of tumor in right lateral wall

M7 section of tumor from right posterior wall

M8 section of normal anterior bladder wall

M9 section of normal bladder dome

M10 section of tumor with right bladder neck and right prostate base

M11 en face resection of the apical urethral margin

M12 M14 sections of right prostate distal to proximal

M15 M17 left prostate sections distal to proximal

M18 right seminal vesicles and vas deferens

M19 left seminal vesicle and vas deferens

M20 longitudinal sections to include posterior trigone, bladder neck, and prostate base

Specimen N is received in formalin and designated ' appendix' and consists of an appendix with attached adipose. The appendix measures 7.0 cm in length and averages 0.6 cm in diameter. The mesoappendix measures 6.5 x 2.2 x 1.0 cm. The serosa of the appendix is mildly hyperemic and otherwise unremarkable. On section, the lumen appears narrowed throughout. No lesions are grossly identified. Representative sections of appendix is submitted, N1.

Microscopic Description

The stain quality is acceptable.

A-N. The microscopic findings are reflected in the diagnoses rendered.

Intraoperative Diagnosis

**** This report is a compilation of information originating from multiple source systems. ****

Printed:

Page: 9 of 10

Pathology Results

Name: _____

MRN: _____

DOB: _____

Sex: _____

Time in: _____ Time out: _____

A1-FS, Right ureter: ' Reactive endothelium. No dysplasia or carcinoma seen.'

B1-FS, Left ureter: ' _____ ureter and reactive endothelium. No dysplasia or carcinoma seen.'

Time in: _____ Time out: _____

C1-FS, Left internal iliac lymph node: ' Eight lymph nodes, negative for tumor.'

Reported to ' _____

Electronically Signed Out

**** This report is a compilation of information originating from multiple source systems. ****